

Prior Authorization Request - Inpatient Admission

Health Plan

Humana Medicare Advantage. Plan ID: H5216-014-000. Request type: standard pre-service review.
Date of request: 2026-10-06

Member Information

Member: Linda Carter DOB: 1980-04-12 (age 46) Member ID: MBR-41001
Address: 214 Maple Court, Dayton, OH 45420
Phone: (937) 555-0101 Sex: F
Primary insurance: Humana Medicare Advantage (primary). Secondary: none on file.

Requesting Provider

Dr. Alan Reyes, MD -- Bariatric Surgery
NPI: 1457382096
Practice: Riverbend Bariatric Surgery Center
Phone: (937) 555-0120 Fax: (937) 555-0121

Requested Service

Requested service: Elective inpatient admission, laparoscopic sleeve gastrectomy (stand-alone)
CPT 43775 ICD-10: E66.01 (Morbid obesity due to excess calories); Z68.41 (BMI 40.0-44.9); E11.9; I10; G47.33
Planned admit date: 2026-11-12
Level of care requested: INPATIENT
Place of service: Hospital, inpatient (acute care).

Submission Contact

Submitted through the provider portal by Sheila Brandt, Authorization Coordinator.
Callback: (937) 555-0120. Supporting clinical documentation attached (6 pages including this request).
Expected length of stay: 1 midnight.
Procedure is planned as a stand-alone laparoscopic sleeve gastrectomy.

Surgical Consultation - Bariatric Surgery

Date of consultation: 2026-09-15. Patient: Linda Carter, age 46, female.

History

Ms. Carter is referred by her primary care physician for surgical treatment of severe obesity. Weight has been above 230 lb for 12 years. She has type 2 diabetes mellitus (diagnosed 2021, on metformin 1000 mg twice daily), hypertension on lisinopril 20 mg daily, and obstructive sleep apnea treated with CPAP. She also reports knee pain with walking.

Measurements

Height 64 in. Weight 245 lb. BMI 42.0 kg/m² (measured in clinic 2026-09-15). Highest recorded BMI 44.0.

Prior medical treatment for obesity

Completed a 6-month physician-supervised weight management program (Riverbend Medical Weight Management) from 2026-02-10 to 2026-08-12 with monthly visits, registered dietitian counseling and a documented calorie-controlled diet. Net weight change over the program: 252 lb to 247 lb. Patient was unable to maintain a healthy weight despite adequate participation. Earlier attempts include a commercial program in 2019 with regain.

Assessment

Severe obesity with obesity-related comorbidities (type 2 diabetes, hypertension, OSA). No history of prior bariatric surgery. Non-smoker. No history of eating disorder. No known cardiac disease. No hepatic disease. No autoimmune disease.

Plan

Laparoscopic sleeve gastrectomy as a stand-alone procedure. No staged operation is planned. Patient has received education on the operation, risks, the required lifestyle changes and lifelong follow-up, and has demonstrated understanding and willingness to carry out these changes. I certify that the patient has made a diligent effort to achieve a healthy body weight, as described in this note and in the attached program records.

Dr. Alan Reyes, MD, Bariatric Surgery. Board certified, American Board of Surgery. Fellowship trained in minimally invasive and bariatric surgery. Completed an accredited surgical residency; maintains continuing medical education.

Medical Weight Management Program - Visit Summary

Riverbend Medical Weight Management. Physician: Dr. Nina Patel, MD. Registered dietitian: Rachel Owens, RD.

Date	Weight	Visit
2026-02-10	252 lb	Intake, 1500 kcal plan, RD counseling
2026-03-10	250 lb	RD follow-up, food log reviewed
2026-04-14	249 lb	MD visit, activity plan added
2026-05-12	251 lb	RD follow-up
2026-06-16	248 lb	MD visit
2026-07-14	249 lb	RD follow-up
2026-08-12	247 lb	Final visit, program completed

Summary: Patient attended 7 of 7 scheduled visits and kept a food log. Weight loss was not sustained beyond 5 lb. Structured dietary program overseen by a physician and a registered dietitian. Patient failed to maintain a healthy weight despite adequate participation.

Psychological Evaluation - Pre-bariatric

Date: 2026-08-28. Evaluator: Dr. Susan Lindgren, PhD, Clinical Psychologist.

Patient has no history of psychiatric disorder and is not taking psychotropic medication. Screening for depression (PHQ-9 score 4) and anxiety was negative. No evidence of binge eating disorder or bulimia. No alcohol or substance use disorder. No current tobacco use.

Patient understands the operation, the dietary changes and the need for lifelong follow-up, and is judged capable and willing to carry out these changes. Cleared from a psychological standpoint to proceed with bariatric surgery.

Primary Care Note - Comorbidity Documentation

Date: 2026-09-02. Physician: Dr. Marcus Hill, MD.

Type 2 diabetes mellitus: hemoglobin A1c 8.2% on metformin 1000 mg twice daily. Hypertension: blood pressure 142/88 on lisinopril 20 mg. Obstructive sleep apnea: AHI 28 on 2025 sleep study, uses CPAP nightly. These conditions are not easily controlled with non-invasive means and pose considerable risk to function and survival if untreated.

Cardiac risk assessment: EKG normal sinus rhythm, no cardiac history. Pulmonary: no COPD. Cleared for general anesthesia, ASA class 3.

Postoperative Care Plan

Postoperative care will be provided by the operating surgeon immediately after surgery and throughout the global period.

Follow-up visits with the bariatric surgery team are scheduled at 2 weeks, 3 months, 6 months and 12 months, which is at least three visits in the first year. Lifetime follow-up for dietary issues, vitamin and mineral supplementation, exercise and lifestyle changes will be supported by counseling and a monthly support group supervised by Dr. Reyes.